

Selective Mutism: An Often-Undiagnosed Disorder

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History and Definition of Selective Mutism:

Selective Mutism is a rare and often-missed disorder that affects many children, teens, and adults. Understanding and identifying Selective Mutism can be incredibly challenging, especially for those who have never even heard of it. To understand this disorder, it is important to understand the history and current definition of Selective Mutism. Priscilla Wong wrote “*Selective Mutism - A Review of Etiology, Comorbidities, and Treatment*” that was published in 2010 in the *National Library of Medicine*. In it she goes into the history of this disorder and the previous misconceptions of it. In 1877 Kussmaul theorized it was a voluntary disorder where people or children refuse to speak and named it “aphasia voluntaria.” It was later renamed in the 1930s as “Elective Mutism” and still operated under the misconception that it was a voluntary disorder. It continued under this misconception until 1994 when it was officially renamed “Selective Mutism” in consideration of the involuntary and selective nature this disorder represents (Wong, 2010).

Most often diagnosed in early childhood, Selective Mutism is defined by *Psychology Today* as “a rare childhood anxiety disorder in which a child experiences a trigger response and is unable to speak in certain situations or to certain people” (para 1, 2021). They go on to explain that this reaction is not voluntary, but that it is an involuntary reaction that subjects find uncontrollable (Psychology Today, 2021). According to the NHS (National Health Services – UK) Selective Mutism affects 1 out of every 140 children and is often described as a phobia of talking to certain people or in certain situations. They talk of how this is a trigger response that causes the subject to “freeze with feelings of anxiety and panic” that makes talking impossible (para 1, 2023).

It is important to note, however, that Selective Mutism does not just affect children. As Dr. Clarissa Gosney pointed out in her article “*Selective Mutism in Teens and Adults*,” without treatment this disorder can follow a child well into adulthood. She goes on to say it is important to realize the extent to which this can have a negative impact on their lives. Anything from a random innocent comment, constructive criticism, any form of teasing, or a perceived judgement can have a lasting and negative effect on their ability to speak or, in situations where they had been speaking, to keep speaking (2023).

Clinical Assessment / Symptoms of Selective Mutism:

As mentioned before, Selective Mutism is often considered a childhood anxiety disorder. As the NHS points out it often starts in early childhood around ages 2 – 4 and is often diagnosed around the time most children start school (para 2, 2023). While there are many significant symptoms unique to Selective Mutism, Dr. Wong points out that evaluations for Selective Mutism take a “comprehensive and multimodal approach” (para 12, 2010). This “multidimensional assessment” combines many factors from information provided by parents and teachers to different health professional assessments (such as psychologists, psychiatrists, and speech pathologists.) Though most often, direct observations of the subject in both clinical and home studies are vitally important. The importance of ruling out any other cause of their mutism such as schizophrenia or autism cannot be overstated (Wong, 2010, para 12).

The National Health Services of the UK gives a breakdown of the common symptoms associated with Selective Mutism. The most common and obvious symptom is eye contact avoidance in social situations. Other common symptoms include being “socially awkward” or being nervous and anxious, they are often considered rude, can be stiff / uncoordinated in

appearance, act withdrawn/shy, or sometimes seen as a stubborn and aggressive individual.

There are various levels of mutism associated with Selective Mutism. Some cannot communicate in any way be it verbal, written, or through gestures. Others can communicate through gestures while some may be able to communicate to someone they trust in a whisper in a specific situation. (para 4, 2023)

The NHS goes on to say the specific parameters they are looking at when diagnosing Selective Mutism. Starting with not speaking in certain social situations but that they can communicate freely in what the subject deems to be safe situations, like at home or in private. The next parameter they are looking for is that their inability to speak in certain situations must exceed 1 month and that their inability to speak must interfere with their ability to function in that situation. The final, and most defining, is that their inability to speak is not caused by any other disorder (2023).

Causes / Etiology of Selective Mutism:

As Dr. Wong points out there are currently seven theories for the etiology of Selective Mutism. The first she mentions is the “Psychodynamic theory” which purposes that their mutism is based off unresolved conflict and can be viewed as a form of a coping mechanism. The second is the “Behavioral theory” and purposes that their mutism is based off learned behavior and can be seen as a “social trigger.” The third theory is “Social phobia and social anxiety” in which they view their mutism as a “variation” of social phobias. The fourth is the “Family systems theory” in which they suspect the basis of their mutism is from a parent’s “excessive need to control their child as well as an associated dependence.” The fifth theory is “Dissociative identity theory” which places the blame of mutism on multiple personalities. The sixth and often “uncommon

explanation” of Selective Mutism is “Posttraumatic stress theory” which purposes that their mutism is a trauma response. The final theory Dr. Wong presents is the “Developmental psychopathology theory” that suggests that those who are predisposed to anxiety encounter situations that result in avoidance of conversations and selective mutism. It is important to remember, as Dr. Wong points out, that these are just theories and are based on small studies that desperately require more in-depth study (paras 35-50, 2010).

Treatment of Selective Mutism:

According to the NHS the earlier you can get a diagnosis the easier it is to treat, those people that remain undiagnosed until later in life often have a longer and more difficult journey ahead of them. They go on to say that the treatment will depend on three main things. The first criteria that treatment is dependent on is the length of time they have gone undiagnosed. The second is the consideration of other anxieties or disorders. The last one is how cooperative their family and educators are in their everyday life. The most critical form of treatment is support both emotionally and physically. The NHS even says, “The need for individual treatment can be avoided if family and staff in early years settings work together to reduce the child's anxiety by creating a positive environment for them” (section 9, para 4, 2023).

For those who go undiagnosed into their later adolescence and into their early adult years, most treatment options are geared towards therapy. These can range through many forms such as cognitive behavioral therapy, graded exposure, stimulus fading, positive and negative reinforcement, and desensitization. The NHS goes on to say that medication should be reserved for those older teens and adults who suffer from other forms of anxiety or depression and that it

is not advised for use to treat children or those who do not suffer from other debilitating disorders. (2023)

Observations:

My younger sister, Lou, battles this disorder every day. Due to the unknown nature of this disorder, she went undiagnosed for most of her youth. It did and still does interfere in her everyday life, making it nearly impossible to participate in school, work, or everyday social situations. Something as simple as going to the grocery store, the doctor, or a casual conversation can be as big an obstacle as climbing Mt. Everest. She dropped out of school at 16 due to the complications she was having in communicating with her teachers. She later went on to get her GED and gain college credits. But this disorder has taken away from her countless experiences that we all take for granted every day. I honestly cannot describe the feeling of watching your sister shut down, I am afraid my words will fail to fully describe the impact it has on her. The vacant, terror-filled eyes and the heartbreak of seeing her struggle for the words she desperately wants to say. Since her diagnosis she has had a better understanding of her triggers and has been able to get the therapy she needs to help overcome it. Thankfully, through her diagnosis my family has learned how to help her as best we can and at age 32, she has come an incredibly far way. Though as the NHS pointed out, the longer it takes to catch it, the longer and more difficult the journey (2023).

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